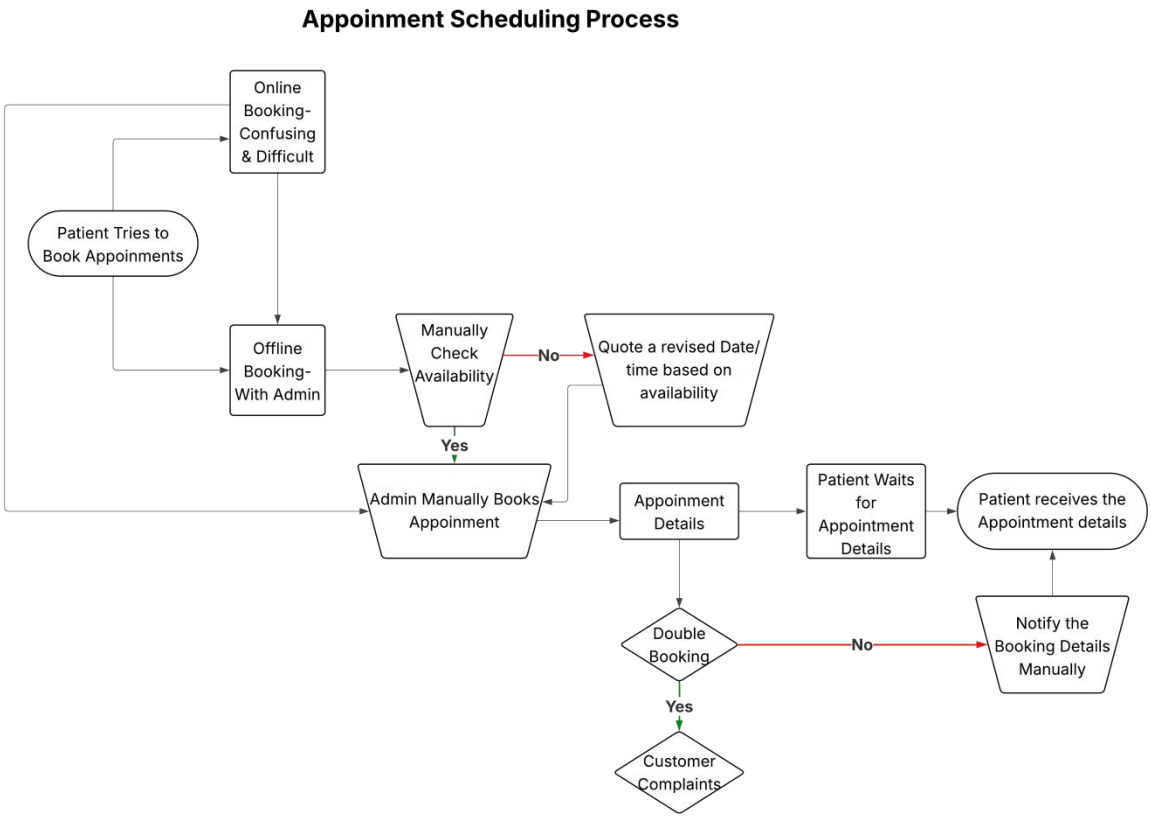
Process Model Analysis and Findings
HealthFirst Care – Process Optimization Initiative

1. Analysis of Current Workflows (As-Is)

The As-Is process models revealed several operational inefficiencies across appointment scheduling, patient check-in, and interdepartmental communication workflows.

1.1 Appointment Scheduling – Key Challenges

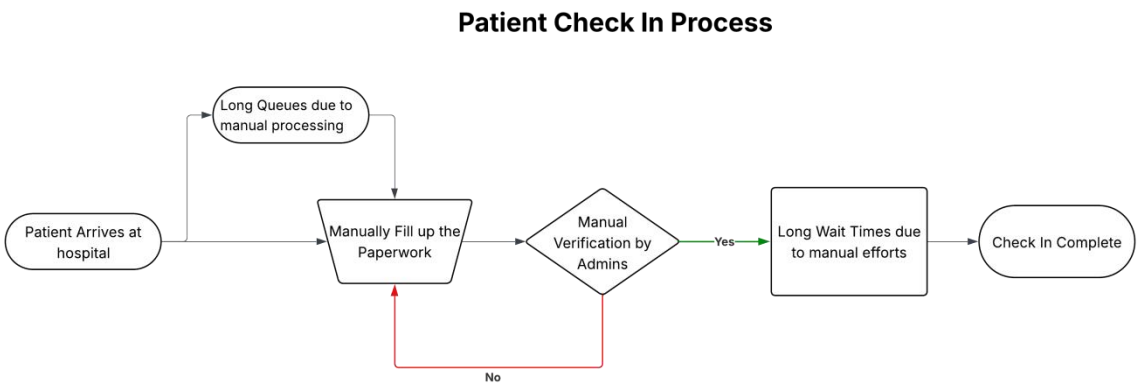


The current scheduling workflow is heavily dependent on manual intervention by administrative staff. Key issues identified include:

- Manual availability checking, increasing processing time.
- Lack of automated conflict detection, resulting in double bookings.
- Delays in notifying patients about booking confirmations.
- Limited real-time visibility into doctor and resource availability.
- High likelihood of patient complaints due to booking errors and communication gaps.

The absence of a centralized, automated system contributes to inefficiencies, operational bottlenecks, and reduced patient satisfaction.

1.2 Patient Check-In – Key Challenges

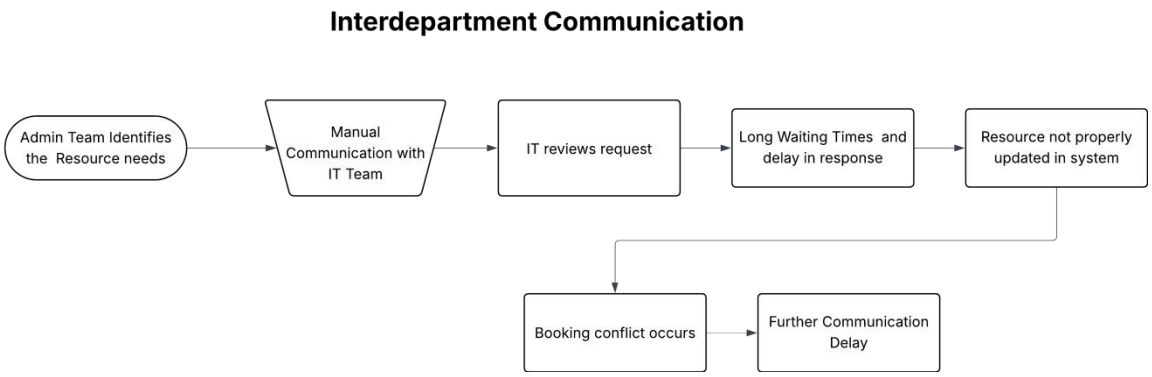


The As-Is check-in workflow highlights significant dependency on manual processes, including:

- Physical paperwork completion.
- Manual verification by administrative staff.
- Long queues caused by processing delays.
- Rework loops due to incomplete or incorrect information.
- Delayed notification to respective departments.

These inefficiencies contribute to longer waiting times and reduced operational throughput during peak hours.

1.3 Interdepartmental Communication – Key Challenges



The current communication process between administrative and IT teams lacks structure and automation. Identified issues include:

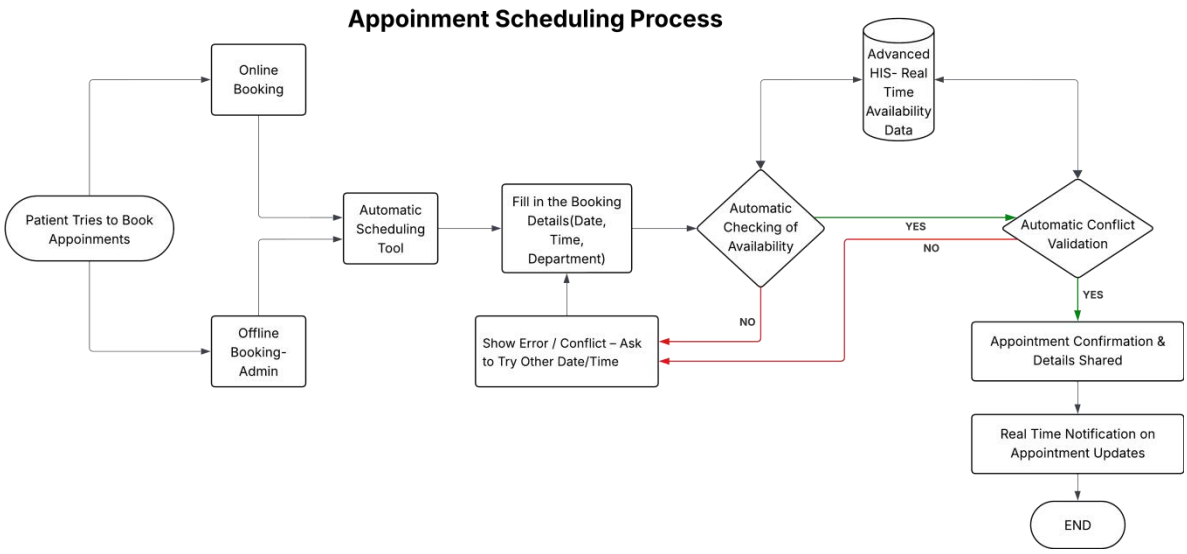
- Manual communication of resource requirements.
- Delayed updates in system records.
- Limited real-time visibility of resource allocation.
- Increased risk of double booking due to synchronization gaps.
- Prolonged resolution time for operational issues.

The fragmented communication structure results in preventable booking conflicts and inefficient resource utilization.

2. Rationale for Proposed To-Be Solutions

Based on the identified inefficiencies, optimized To-Be process models were designed to introduce automation, real-time integration, and centralized coordination.

2.1 Automated Appointment Scheduling



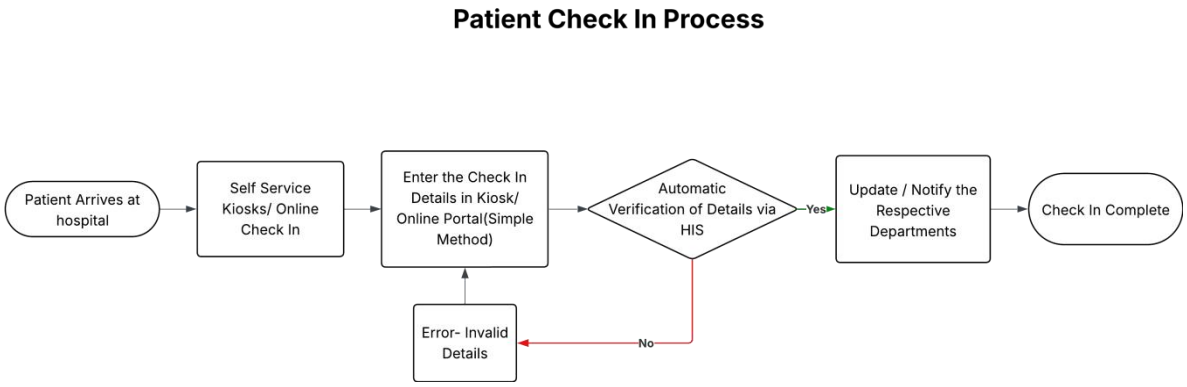
The proposed To-Be model introduces:
An automated scheduling tool integrated with the Advanced Hospital Information System (HIS).

- Real-time availability checking.
- Automatic conflict validation.
- Instant appointment confirmation.
- Real-time notification via digital channels.

Expected Impact:

- Reduction in double bookings.
- Faster appointment processing.
- Improved resource visibility.
- Increased patient satisfaction through timely updates.
- Reduced administrative workload.

2.2 Streamlined Patient Check-In



The optimized check-in workflow includes:

- Self-service kiosks or online check-in portals.
- Automated real-time verification via HIS.
- Instant notification to respective departments upon successful check-in.
- Digital error handling and correction loop.

Expected Impact:

- Reduction in long queues.
- Decreased manual workload for staff.
- Faster patient processing.
- Improved data accuracy.
- Enhanced patient experience.

2.3 Structured Interdepartmental Communication

Interdepartment Communication

The proposed solution introduces:

- A centralized dashboard tool for raising resource requests.
- Automated IT notifications.
- System-driven approval workflows.
- Automatic resource booking and blocking to prevent double allocation.

Expected Impact:

- Improved communication transparency.
- Faster response times.
- Elimination of booking conflicts due to visibility gaps.
- More efficient resource utilization.
- Reduced operational delays.

3. Overall Expected Operational Improvements

The transition from manual, fragmented workflows to automated and integrated systems is expected to result in:

- Reduced patient wait times.
- Improved scheduling accuracy.
- Increased operational efficiency.
- Enhanced data synchronization across departments.
- Improved stakeholder coordination.
- Higher patient satisfaction and service reliability.

The To-Be process models directly address the inefficiencies identified in the As-Is analysis and align with HealthFirst Care's objective of improving patient experience and operational performance.

4. As-Is process Models

AS_IS_HEALTHFIRST_CARE - Page 1.pdf

5. To-Be process Models

TO_BE_HEALTHFIRST_CARE - Page 1-2.pdf